SURGICAL PATHOLOGY REPORT

Kallang Medical Centre
Department of Anatomical Pathology
SURGICAL PATHOLOGY REPORT

Patient: Nurul Huda binte Osman NRIC: G4835030K
DOB: 19/04/1968 MRN: MRN4539704
Phone: +65 8242 5040
Accession No: PA26-95098 Collected: 15/02/2026 Reported: 21/02/2026
Requesting surgeon: Dr Mohd Abdullah

Surgica. CLINICAL HISTORY: Left breast mass, positive biopsy. Rule out breast CA. GROSS EXAMINATION: A. "Left breast biopsy", in formalin. Received is a portion of adipose. tissue which measures 4.5 x 4.0 x 1.9 cm in greatest dimensions. A firm. nodule is felt within the adipose tissue. Sections through this nodule reveal. a white, gritty mass with irregular borders which measures 3.0 x 1.3 x 2 cm in. greatest dimensions. The tumor approaches the margin of resection. Frozen. section was done on this tumor. Initially, the tissue is submitted for ER/PR. receptors. The frozen section remnant is submitted in Block A1. Additional. representative sections of tumor are submitted in Blocks A2 and A3. B. "Left breast with axillary contents". Received is a 620 gram, 20 x 16 x. 3.5 cm modified radical mastectomy with attached axillary tail. The attached. axillary tail measures 8 cm x 7.5 cm x 1.8 cm. The skin ellipse measures 20. cm in length x 7.5 cm in width with a medially eccentrically placed nipple. Lateral and inferior to the nipple is a 5 cm sutured incision. The deep. surface is painted with blue ink. Deep to the incision there is a hemorrhagic. biopsy cavity which measures approximately 5 cm x 4.5 cm x 3 cm in greatest. dimensions. The biopsy cavity approaches the deep margin of the specimen, but. no residual tumor is identified. Along the superficial edge of the biopsy. cavity is a residual firm nodule which measures 1.5 x 2.0 x 0.8 cm is felt. Examination of the remainder of the breast reveals only firm, white breast. tissue and adipose tissue. The axillary tail is examined for lymph node. candidates. Block Summary: B1: Representative sections of nipple. B2: Representative sections of skin through incision. B3-B5: Representative sections of biopsy cavity to include deep margins. B6,B7: Nodule along superior aspect of biopsy cavity. B8: Representative section of upper inner quadrant. B9: Representative sections of lower inner quadrant. B10: Representative section of upper outer quadrant. B11: Representative section of normal lower outer quadrant. B12,813: Lymph node candidates from Level-I. B14-B16: Lymph node candidates from Level-II (a single large lymph node. candidate is bisected and submitted in toto in Block A16) . B17,818: Lymph node candidates from Level-III. INTRA OPERATIVE CONSULTATION: "Left breast biopsy": AF1 - Carcinoma. DIAGNOSIS: A. "LEFT BREAST BIOPSY": INFILTRATING DUCTAL CARCINOMA (3 CM DIAMETER) , NSABP NUCLEAR GRADE 3 of 3. HISTOLOGIC GRADE III: A SPINDLE CELL COMPONENT IS PRESENT. FOCAL INTRADUCTAL CARCINOMA OF COMEDO TYPE. B. "LEFT BREAST WITH AXILLARY CONTENTS": INFILTRATING DUCTAL ADENOCARCINOMA, NSABP NUCLEAR GRADE 2 to 3; HISTOLOGIC. GRADE II. 1 of 2. SURGICAL MARGINS ARE FREE OF MALIGNANCY. ZERO OF 16 LYMPH NODES ARE POSITIVE FOR METASTATIC DISEASE. 2 of 2.

COMMENT: Results discussed by phone with Dr Mohd Abdullah on 21 Feb 2026.

Electronically signed by Dr James Garcia, Consultant Pathologist, on 21/02/2026